

ROYAL HOSPITAL
INFECTION CONTROL AND PREVENTION
SUMMARY OF SCREENING POLICY

Type of organism	MDRA/CRE Multi-Drug Resistant <i>Acinetobacter Baumannii</i> / Carbapenem Resistant <i>Enterobacteriaceae</i> .	MRSA Methicillin Resistant <i>Staphylococcus aureus</i> .	VRE Vancomycin Resistant <i>Enterococci</i> .	C.Auris Candida Auris
Indications for Screening	- All cases which had history of admission in other hospital within past 3 months. - All cases coming from abroad hospitals within 6 months. Swabs: Throat and Perineum+ risk factors*.	- All cases previously admitted to another hospital within past 3 months. - All cases been to abroad hospitals within 6 months. - Undergoing liver or cardiac surgery (preoperatively) or receiving continuous ambulatory peritoneal dialysis (CAPD) or hemodialysis. - Previously MRSA positive and decolonized. Swabs: Axilla, nose & groin+ risk factors*.	- Admitted to Oncology ward. - Admitted to nephrology ward on Hemodialysis. - Known previously VRE positive. *Rescreening is done monthly for chronic frequently admitted oncology and renal patients Swabs: Throat & Perineum+ risk factors*.	- Screen patients hospitalized in hospitals abroad. - Screen patients coming from other hospitals within the last 4 weeks. NB: 3 negative samples are required to ascertain patient negative Candida Auris screening. Swabs: axilla & groin+ risk factors*.

NOTE:

All Pt admitted in critical unit to be screened for (MRSA, MDRA/CRE and C.Auris) on admission.

*Take swabs from wound, urinary catheter, tracheotomy, all sites of central lines; if present along with body swabs.

*All screening should be done when patient is off antibiotics or stopped for 48hrs.

***For C.Auris:**
Screening can be sent as long patient is not on antifungal.